Conservative

- Assurance
- Improvement of nutritional status
- Pelvic floor exercises.

Nursing management

- Assess the blood loss
- Assist and teach the pain relieving techniques
- Educate about hormonal therapy
- Advice calcium rich diet to prevent osteoporosis
- Advice to consult doctor in case of bleeding per vagina during Hormonal therapy
- Advice to do yoga and meditations



Increased risk of pregnancy complications in the presence of fibroids, such as first trimester bleeding, breech presentation, placental abruption, and problems during labor. Fibroids have also been associated with an increased risk of cesarean delivery

2.24 Menstrual Disorders

Menstrual cycles often bring about a variety of uncomfortable symptoms. Menstruation can be either too heavy or too light, or complete absence of a cycle.

Disorders of cycle length

Oligomenorrhea –Infrequent menstruation

Polymenorrhea - cycles with intervals of 21 days or fewer.

Amenorrhea -absence of a menstrual period in a woman of reproductive age.

Disorders of Flow

Hypomenorrhea - light or scantly menstrual blood flow.

Menorrhagia - abnormally heavy and prolonged menstrual period.

Metrorrhagia - bleeding at irregular times, **Menometrorrhagia** (meno = prolonged, metro = uterine, rrhagia = excessive flow/ discharge) prolonged or excessive uterine bleeding occurs irregularly and frequently.

Dysmenorrhea - cramps or painful menstruation

Causes

- Uterine abnormalitites (Fibroids)
- Sexually transmitted diseases (gonorrhea)
- Steroid consumption
- Chronic pelvic diseases
- Immaturity of the hypothalamic-pituitary axis
- Polycystic ovarian disease

Psychological factors

- Anxiety
- Stress
- Emotional trauma;
- Drugs
- Drug addiction
- Steroid administration
- Metabolic or endocrine diseases
- Nutritional deficiency

Peripheral causes:

- Ovarian cyst,
- Pelvic tumors

Diagnosis

- General examination
- Abdominal and pelvic examination
- Transvaginal ultrasound
- Dilatation and Curettage
- Hysteroscopy

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